

Medicaid, Political Life, and Fragmented Democracy

“No one can travel the length and breadth of the United States without the conviction of its inexpugnable variety. East and West, South and North, its regions are real and different, and each has problems real and different too. The temptation is profound to insist that here, if ever, is the classic place for a federal experiment ... This kind of argument is familiar in a hundred forms. I believe that, more than any other philosophic pattern, it is responsible for the malaise of American democracy.”

– Harold Laski, 1939

“I’ve worked in Ohio, I’ve worked in California, I’ve worked in Washington and as a single parent I have always had Medicaid. Now here in Georgia in 2012 they cut me off because I have a part-time job ... This is the only state that has ever cut me off Medicaid because I have a job, a part-time job that does not allow me to afford insurance. Why cut off someone for that? You know I have to be healthy to work right? You want me to be a functioning, self-sufficient adult. So why take away the very thing that keeps me functioning, self-sufficient, and able to provide?”

– Terrie, 2012

One warm August day I sat in a burger joint on the outskirts of Atlanta and spoke with Terrie, a middle-aged black woman.¹ I treated Terrie to a banana milkshake and she gave me the scoop on Medicaid. Because of her limited income, Terrie had been on and off Medicaid for more than seventeen years. Over this period of time she lived in Ohio, California, Washington, Illinois, and Georgia. Of everything she divulged about her experiences, Terrie’s most emphatic observation was that Medicaid varied dramatically from place to place. Before we sat down to talk, I told her that the purpose of my research was to understand what it

was like to be insured through Medicaid. Then, I began with a very open question: "Tell me a little about yourself?" In response, Terrie portentously broached the core issue that she would dwell on throughout our conversation:

My name is [Terrie] and I have a sixteen-year-old son; he'll be seventeen soon. I have traveled a lot so the difference between state to state with Medicaid and what it offers and the programs and how consistent they are; I have a lot of experience with that. Being in [Medicaid] seventeen years, you know, it has just been a whirlwind with keeping [my son] safe and healthy.

I was struck by the prominence of geographic variation in Terrie's first words. Her emphasis on this proved persistent and pointed. She soon offered details about the merits of various state Medicaid programs: "Ohio is the easiest, they do care about their people." "California, their process is probably faster, but there are so many people and it's so rapid that it is out of control." "In Georgia, there are limitations in everything that they offer ... you either can get this and can't have that, or you can have this and can't get that ... you can only go to this doctor on this day at this time."

Foremost on Terrie's mind was how the intergovernmental design of Medicaid affected interstate travel. She shared this:

When I knew I was going to meet you, I got upset a little bit thinking about it, because I've got a lot to say about Medicaid. Like, for instance, my grandmother was here from Chicago just this past week. She went to the doctor and to the hospital. We got some prescriptions we needed to fill for her. So we go to the pharmacy and we can't fill this prescription because Medicaid is non-transferable state to state ... and her prescription was \$190, so we really had to find \$190 for her prescription. That was amazing ... and for something that's provided by the government ... you're limiting the use of something meant to make people better.

As our conversation progressed, Terrie elaborated on the (state-specific) political lessons that she drew from such geographic discontinuities:

If it was about helping people, you would say yes, let *my state* be more productive and healthy so that we do not have people losing their lives [and] so that they can be productive citizens ... These types of people are here serving you food when you go out ... wouldn't you like to know that they are healthy? These are the people that you want to give Medicaid, the very people who are serving your food ... you do not want to insure the very people that are serving you food? (emphasis added)

Toward the end of the interview, I asked Terrie whether politics might be an avenue for changing some of the things we had talked about. She stared at me in what seemed like disbelief and sardonically uttered, “white noise.” I looked back at her, confused. Again, she reiterated, “white noise.” After a few seconds of awkward silence, I asked her what she meant. She explained by saying,

White noise is the people that choose to say well, if they give it to me, they give it to me, if they don’t, they don’t ... white noise also means that you feel like in your world, *you have no say, no say in the process if you don’t agree with what is going on in Medicaid*. It’s demeaning, you know, the process. So when you want to, you can’t get through that door and you are wondering how can you possibly get through the magic door to get people to understand ... I have never seen anyone really stand up about Medicaid. We fight a little bit ... politically we fight a little over welfare reform and Medicaid reform and all that, but in general as people, I don’t know why we don’t fight (emphasis mine).

Terrie was among those who did not fight. Most of her time was spent working grueling low-wage jobs while taking care of her son, and she simply did not believe Georgia was a place where one could make a difference on issues like Medicaid. Throughout our conversation, she would recount some feature of how Georgia’s Medicaid program operated, compare it to another place she had lived (often Ohio), sigh audibly, and say, “that’s Georgia.”

Terrie recognized that Georgia was not the only “headache when it comes to Medicaid.” She named other culprits, like Louisiana, a state that “doesn’t give a toss about Medicaid whatsoever.” She thought this was an injustice, reasoning that “Everyone in America needs to be covered so America is covered.” Still, Terrie saw little hope for change. She was mostly resigned to a life of second-class citizenship. She even admitted being “amazed” that I cared enough to talk to her, much less write an entire book about people on Medicaid.

Terrie’s contextually inflected experiences with Medicaid were accompanied by feelings of political inefficacy and powerlessness, “white noise” as she dubbed it. She was not exceptional in this regard. Though the details differed in rich and revealing ways, nearly all of the Medicaid beneficiaries I interviewed for this book described program experiences that varied across states, counties, and even neighborhoods – critically shaping political life along the way. John, Fiona, and Daphne provide additional examples. Before I proffer a single word about academic literature or scholarship, I present their stories. I lead with the voices of

Medicaid beneficiaries because their experiences paint a compelling and multitoned portrait of the largely veiled operations of federalism in the everyday lives of economically and (disproportionately) racially marginal Americans. The ensuing narratives exemplify the profound implications of federalism for democratic citizenship and provide a springboard for the scholarly inquiry taken up in this book.

JOHN: “MARRIED” TO MICHIGAN MEDICAID

John is a white man from Michigan who was diagnosed with a life-threatening chronic illness in early childhood. He expressed gratitude for Medicaid, but he also admitted feeling as though he was “married to Michigan” medically, without “the option of really going other places.” Like Terrie, John described an out-of-state trip gone wrong: “I actually hurt my leg and I got like a patch of gout down in my ankle and [Michigan] refused the bill and I got a whopping bill in the mail that I am still paying.” Further underscoring his weddedness to Michigan, John discussed his dashed hopes of relocating to Arizona to start a new life.² After speaking to caseworkers and friends in Arizona, John learned that the Arizona Healthcare Cost Containment System (that is the name of the state Medicaid agency) had meager home health care provisions that would not cover the services he required. “I got some other friends [who] live down [in Arizona] who have the same disease I do and ... they’re stuck over there with what they got and it just kind of becomes a struggle.” Reluctant to jeopardize his life-sustaining benefits, John decided not to move. He explained that “beggars can’t be choosers,” and declared that he would “live on Mars” if he had to. John also pointed out that if the government “just did like a federal insurance and everybody had their insurance card regardless of who we are, where you are, I think it would solve a lot of problems for people, but I don’t think that’s going to happen.” Incredulous about the possibility of change, John noted that Medicaid beneficiaries were politically lackluster because “it does kind of feel like you just reach a fork in the road where you just give up, you just lose.” Throughout our conversation, he continually referenced his thwarted aspirations: “I wanted to venture off and go try to do something different for myself ... seeing other family and friends and they’re able to move on ... it is tough.” When I finally asked him what he thought about “politics,” he assured me that he was not the least bit “interested in that.” I return to John in

subsequent chapters because it turns out that when mobilized by a cause proximate to his life, health, and community, he cares quite a bit about “politics” (which, as I make clear throughout this book, extends far beyond voting and elections). Still, separate from the concrete political outcomes I detail later on, the snapshot of John offered here speaks to how profoundly a federal political system conditions life for those who find themselves in need of assistance from the government to secure vital resources.

FIONA: “MEDICAID SAVED MY BABY’S LIFE”
(BUT ALMOST DIDN’T)

Fiona’s story was more heartening, but the challenges she encountered are nonetheless instructive. Fiona was unexpectedly thrust into the world of health care policy when her son Jack was born with a potentially fatal tumor on his leg. With her partner in school, the family lost health coverage when she quit her job to care for Jack. Sympathetic and supportive hospital employees in North Carolina helped to sign Jack up for Medicaid. The program paid for everything and even covered expenses retroactively from a period before he was enrolled. Fiona had great things to say about Medicaid because it met her family’s most dire need during a perilous time. Nonetheless, she recounted a disconcerting problem. After about a year, the treatment that Jack’s physicians in North Carolina were providing stopped working. Fiona sought care from experts in Boston who were more knowledgeable about the rare condition that Jack suffered from. But North Carolina’s Medicaid program refused to cover that care, cautioning Fiona that if she pursued treatment in Massachusetts, she would have to do so on her own dime. At a devastating loss, Fiona waited and hoped. She soon noticed that Jack’s tumor was shrinking. Doctors said that the treatment he received in North Carolina had kept him alive long enough for his body to mount its own defense against the illness. Fiona avowed, “Medicaid saved my son’s life!” I relished in her happy ending, but wondered what would have happened if Jack’s body had not taken over. Fiona rightly spent little time pondering this. Instead, her experience with Medicaid motivated a new career: she now works for a grassroots organization advocating “health care for all.” In this role, Fiona coordinates with activists in Florida, Texas, and North Carolina to spearhead the fight for Medicaid expansion in the states that have refused it.

DAPHNE: MEDICAID AS THE BURGER KING OF
THE HOOD

At the time we initially spoke, Daphne – a young black woman in her early twenties – had been insured through Medicaid for her entire life. She grew up in a high-poverty, high-crime, hyper-segregated neighborhood in Syracuse, New York. She was living at home, attending community college, and desperately trying to “get out of the hood.” Her life had been so deeply defined by where she lived, that when I asked her about Medicaid, the overbearing power of place lurked in the sub-text of nearly all that she relayed to me. For Daphne, Medicaid meant the scary clinic downtown where only the most indigent beneficiaries ventured. It meant being treated in “ridiculous” ways by health care practitioners who surely would not do the same to (white) people from nicer neighborhoods like “Fayetteville or Cicero or North Syracuse.” Medicaid meant missing out on high-quality care, not (only) because the program itself was inadequate, but because the *places* where poor people utilized the benefits were lacking. As far as the practical application of Medicaid services, Daphne told me that “it is different in different places ... [like] say if you’re at an Olive Garden or you go to a Burger King, they treat you really different.” Daphne and those who lived in communities like hers were relegated to the medical equivalent of the cheapest junk food.

Daphne was savvy enough to know that this was politically meaningful. Her politics – like most politics, perhaps – were local.³ No matter the direction our conversation veered, the specter of the neighborhood loomed. What’s more: Daphne mapped the local to places beyond it, and she developed her ideas about policy and politics accordingly. For example, when I raised the topic of Medicaid expansion, Daphne confessed that she was glad New York had expanded the program, but deeply uncomfortable with that decision being left in the hands of any particular state:

I don’t really like the state choosing things ... *I don’t really trust the state* ... I just think everybody having the same access and it being the same everywhere ... would be more helpful instead of having all these rules and here and there ... I wish it was just nationwide ... I just wish it was the same nationwide and not just the state, because *I don’t think the state could be trusted, honestly. We can’t even trust our police force* (emphasis added).

At the root of Daphne’s mistrust of the state, there was something more proximate: her mistrust of local police. Extrapolating from one level/

institution (neighborhood/police) to another (state/Medicaid), Daphne deduced that policy decisions should be uniform across the nation. In doing so, she unknowingly foreshadowed her own struggles.

A few months after our initial interview I caught up with Daphne and learned that she had come to experience the consequences of federalism more acutely than she ever wanted to. Her long-held dream had come true: she was accepted to a college in North Carolina and given sufficient financial aid to actually go. Exultantly, she had made it out of “the hood.” Distressingly, her Medicaid could not go with her. Since North Carolina had stricter eligibility criteria, Daphne could not qualify and had to remain signed up for Medicaid in New York. Her plan was to visit her doctors when she came home over breaks. So when she tore a major muscle in an accident, she could not immediately see a doctor in North Carolina. Instead, she aggravated her injury by waiting until she could arrange travel home for treatment. As Daphne finished her first year in a bachelors degree program, her move south represented her purest aspirations for upward mobility, but it imperiled her health.

Political Life: Participation and Citizenship

The political lives of Terrie, John, Fiona, and Daphne bear the imprint of Medicaid’s federated structure. By invoking political “life” in this way, I am being intentionally capacious. I mean for this to include “political behavior” as traditionally understood by political scientists (which encapsulates activities such as voting, contacting an official, and protesting). Going further, political life also involves more mundane actions taken by denizens seeking resources, redress, or protection from national, state, or municipal governments (such as filing a complaint within a local bureaucracy or requesting a fair hearing). Extending beyond the realm of participatory action, political life is also about how a person experiences democratic citizenship. Citizenship includes the “rights, duties and obligations imposed by government, as well as citizens’ responses to them” (Mettler and SoRelle 2014: 156).⁴ Folks who cannot recall participating in any political activity at all nonetheless have political lives worth recognizing. Some of the people featured in this book exemplify that. They returned only blank stares when I asked them how, if at all, they “participated” in politics. But when I probed their experiences with Medicaid, they surprised themselves by having more to say than they had thought possible. And despite their disconnection from “politics” in the formal sense, they captivantly articulated

how they were actually *living* politics off the record. This is what Terrie, John, Fiona, and Daphne voice to us. While I will demonstrate why and how people like them take political action, I will also describe how they *live in and make sense of* a polity that is fragmented by the powerful force of federalism.

This book thus illuminates a pressing question: *how do Americans understand and respond to a political system that confers (or withholds) access to resources for the most indigent – not on the basis of needs or rights – but on the basis of geographic location?* To tackle this inquiry, I investigate whether, how, and under what conditions Medicaid influences political life. I find that it has varied effects across states, counties, and neighborhoods. In this way, federalism produces geographically differentiated political capacity across its population of beneficiaries and federalist³ social policy is a key purveyor of political inequality.

Why Medicaid?

The design of Medicaid is one reason for its central place in this book. The intergovernmental configuration of the program allows for wide discretion across state and local levels, which facilitates the policy fragmentation that I seek to understand. Still, Medicaid is not merely a useful case for investigating larger questions; it is substantively important in its own right.

Medicaid is the largest source of public health insurance in the United States and the primary mechanism for providing health coverage to low-income Americans. It is the third most costly domestic program in the federal budget (following Social Security and Medicare) and the biggest source of federal revenue in state budgets, accounting for one out of every six dollars spent on health care (Paradise 2015; Snyder and Rudowitz 2015). Recognizing Medicaid's immense significance, scholars have studied it carefully. Among other things, they have found that Medicaid has (positive) effects on mortality, mental health, financial security, and educational outcomes (Baicker et al. 2013; Cohodes et al. 2016; Finkelstein et al. 2012; Sommers, Baicker, and Epstein 2012a). In this book, I assess its effects on democratic citizenship and political participation.

With growing ensemble of academics, pundits, and ordinary people stridently pronouncing (and denouncing) the economic stratification of political life in America, tracing the conduits of political inequality remains a first-order task. Given that charge, Medicaid is especially

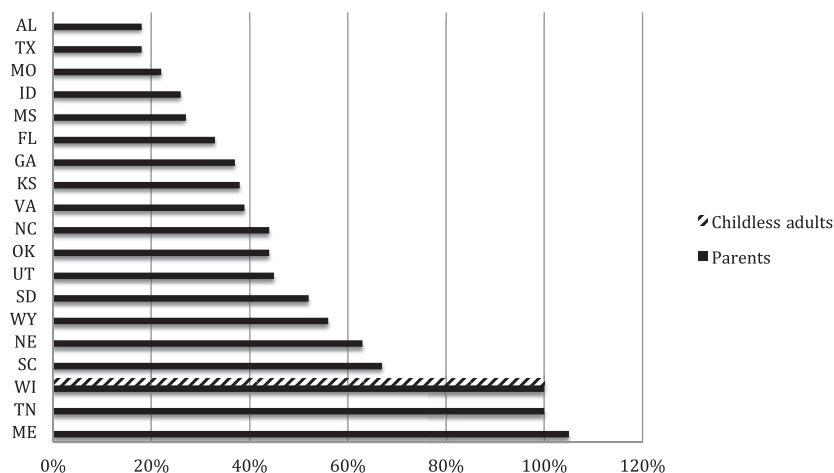


FIGURE 1.1 Income eligibility limits for adults in non-expansion states (2016)
Data from Henry J. Kaiser Family Foundation, 2016

significant. Its beneficiaries are overwhelmingly poor, disproportionately people of color, and unduly prone to health troubles. As such, Medicaid policy brings government directly into the lives of the most marginal citizens (Hernández-Cancio, Bailey, and Mahan 2011; Manchanda 2011).

Foremost among such persons are the economically marginal. Income is the chief criterion of Medicaid eligibility, especially for able-bodied adult beneficiaries.⁶ In the wake of the 2010 Patient Protection and Affordable Care Act (commonly called the Affordable Care Act, or the ACA), non-elderly adults became eligible for Medicaid in states that implemented expansion so long as their income fell below 138 percent of the federal poverty line (FPL). In Wisconsin (a state that did not expand Medicaid) the cutoff is 100 percent of the FPL. The remaining (non-expansion) states have varied income requirements that mostly exclude non-disabled adults; when parents with dependent children qualify, they must fall (on average) below 44 percent of the FPL (see Figure 1.1).

The larger point is this: though Medicaid plays many important roles in our health care system,⁷ the overall picture suggests that it is a primary resource for those who are living in or near poverty. In 2015, more than 90 percent of non-elderly beneficiaries were either poor or low-income: 54 percent were officially below the FPL (accounting for

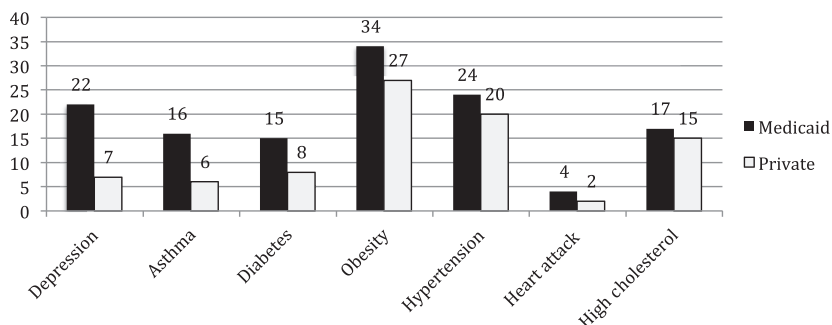


FIGURE 1.2 Percent of Medicaid insured and private insured with health conditions

Data from on Gallup-Healthways Well-Being Index, 2013

36 percent of all Americans in poverty), while an additional 38 percent hovered between 100 percent and 199 percent of the FPL (comprising roughly 30 percent of all low-income Americans).⁸

The tethering of class to race in the United States means that the penalty of beneficiaries has racial repercussions. People of color are disproportionately represented in Medicaid: 32 percent of African-Americans and 34 percent of Latinos were enrolled in 2015 compared to only 16 percent of whites. Though blacks and Latinos (combined) composed 30 percent of the U.S. population in 2015, they accounted for 50 percent of Medicaid enrollees.

Not surprisingly, given well-documented racial and economic health disparities, Medicaid beneficiaries are also more likely to suffer from illnesses like diabetes, asthma, obesity, high cholesterol, and high blood pressure (Mendes 2013). Figure 1.2 contrasts people insured through Medicaid to those with employer-sponsored insurance.⁹ The ailments of beneficiaries are particularly worrisome in light of mounting evidence that health can affect both electoral participation and democratic representation (Gollust and Rahn 2013; Pacheco 2013; Pacheco and Fletcher 2015; Pacheco and Ojeda 2015; Schur et al. 2002).

The trifecta of race, class, and health puts Medicaid beneficiaries among the most politically vulnerable persons in the country. Discerning the democratic corollaries of Medicaid is therefore essential for an accurate picture of political (in)equality in the United States. Relatedly, comprehending the political consequences of Medicaid policy requires attentiveness to its defining institutional facilitator: federalism (more on that in Chapter 2).

What's Democracy Got to Do with It?

You may have noticed that I just shifted from describing the demographic profile of Medicaid beneficiaries (race, class, health) to asserting its importance for democracy. One reason for this jump is that demographic realities signal political vulnerability. When those most affected by a policy are also most marginal in the polity, it is worth stopping to think about how policy and polity interact (Schneider and Ingram 1993). Even more crucially, turning to democracy is unavoidable as we move away from the hard numbers of Medicaid and toward the people behind them. As of 2017, more than 70 million Americans had health coverage through Medicaid. Fifteen percent of those beneficiaries were disabled, 9 percent were elderly, 48 percent were low-income children, and 27 percent were low-income adults. These statistics are useful, but incomplete unless we relate them to the very real human beings who rely on Medicaid to meet their most pressing needs: the jobless mother who is too proud to apply for food stamps but humbled enough by debilitating back pain to embrace Medicaid; the minimum wage-earning father who was kicked off the Medicaid rolls despite his epilepsy and eventually had a seizure that caused him to bite off a portion of his own tongue; the homeless man who insisted that “Government totally sucks. They can determine when you can live and how you can live. They give you that assistance and it determines your lifestyle ... your weight and your health and all those things.” These examples are drawn from the accounts of Medicaid beneficiaries I interviewed for this research. Before talking to them, I could have opined on the “politics of Medicaid” without once mentioning or thinking of an *actual* person. Afterward, I can do no such thing.

Though Medicaid beneficiaries are key stakeholders in the American health care system, many of them hardly make a blip on anyone's political radar. Certainly, some groups of beneficiaries – like the elderly and the disabled – are better positioned politically.¹⁰ But overall, most remain disempowered in the face of polarized, pivotal public debates that may literally determine whether they live or die. They are the numbers behind the statistics in these debates, but where are their voices? They are the people who will be most deeply affected by the policy that politics creates, but what of their political lives?

This is where democracy comes in. The relative quiescence of Medicaid beneficiaries is not an inevitable or inflexible outcome. For example, in September 2011, numerous “My Medicaid Matters” rallies were held throughout the country and thousands of people showed up to oppose

program cuts. The flow of protests continued thereafter. In 2012, demonstrators in Texas held rallies challenging the state's defunding of a women's Medicaid program and warned Governor Rick Perry not to "mess with Texas women." In 2013, activists in Philadelphia demanded that then Governor Tom Corbett expand Medicaid, holding up signs with the query, "Medicaid or Millionaires, what will it be?" In 2014, two dozen people converged on Topeka's copper-domed state house, waving American flags and urging Kansas Governor Sam Brownback to implement Medicaid expansion. In 2015, hundreds of protesters descended upon the Missouri state capital with the goal of letting "the legislature and senators know that ... there are many people in the state of Missouri [who] are dying that have no means to go to a doctor and to get assistance."¹¹ On Valentine's Day in 2017, hundreds of activists in North Carolina called for lawmakers to "have a heart," delivering thousands of individually signed "Valentines" to two Republican senators asking them to expand Medicaid and protect the ACA.¹²

Just over a month later, on March 21, 2017, Rise and Resist, a direct-action political group, organized a rally in New York City around the theme "Medicaid Block Grants Kill."¹³ Roughly 300 demonstrators gathered in front of Federal Plaza at the intersection of Broadway and Worth, holding signs that read, "Medicaid Block Grants Kill," "Medicaid Cuts Kill," "We Need Medicaid," and "TrumpCare Makes Us Sick." (Some of these protestors are pictured on the front cover of this book.) Medicaid beneficiaries in attendance shared about why they needed the program; they expressed their disdain for President Trump and they literally pleaded for their lives. Foremost among their concerns was the prospect that Medicaid budgets would be slashed and thousands of lives sacrificed at the altar of "state flexibility." Ironically, on the same day as the rally in New York City, Congressman Bruce Westerman (R-ARK) published an op-ed explaining how "Medicaid Block Grants Give States More Freedom" and arguing in favor of them.¹⁴ The divergence between Westerman's sentiments and those of the Medicaid beneficiaries fighting for their lives on the streets of New York City bespoke a gaping chasm between the elites making decisions about Medicaid policy and the everyday Americans affected by those decisions.

Since the passage of the ACA, hundreds of rallies about Medicaid have taken place, attended by anywhere from 25 to 8,000 people (at a single event).¹⁵ Though scattered and sporadic, such activity hints at the life that is coursing through the veins of our body politic. It insinuates the potential for Medicaid beneficiaries (and would-be beneficiaries) to play an

active role in American democracy. However, the political capacity of this population and the quality of their democratic citizenship depend upon processes that are structured by the institution of federalism (Mettler 1998; Soss, Fording, and Schram 2011). In the next chapter, I take a closer look at federalism: *what is it and why is it so important?* But first, I provide a brief mapping of this book, say a bit about my methodological approach, and clarify the political stakes of this work.

Outline of Book

Since federalism is a foundational American institution that dynamically structures politics at multiple levels of governance, across various repertoires of action, and with dramatic material and ideational consequence, this book covers a broad scope. After conceptualizing federalism in more detail, Chapter 2 situates this book in a broader literature and poses a framework for understanding my contribution to scholarship on political participation, policy feedback, and contextual effects.

Chapter 3 then pans out, with a historical perspective on the profound implications of federalism for U.S. social welfare policy – most especially health policy and Medicaid. It has always been the case that what it means to live in poverty in Maine is very different from what it means in Michigan; that Cook County (IL) offers a markedly different array of services than Cuyahoga County (OH); and that teetering at the economic brink in Seattle is quite unlike doing so in Syracuse. I show how such incongruences have been buttressed and amplified by federalism. Medicaid is an especially arresting case, epitomizing how federalism generates inequity by creating geographic variation in access to vital resources.

After recounting this historically, I present a contemporary sketch of Medicaid to demonstrate how federalism enables systematic disparities among economically and racially marginalized Americans. This is not because federalism is inherently bad. Federated political systems have a host of well-noted advantages and disadvantages, so coarse characterizations will not do (Zimmerman 2008). Nor is Medicaid the problem; it is a program that saves thousands of lives every year and provides tens of millions of Americans with the security of health insurance (Sommers, Baicker, and Epstein 2012a). Chapter 3 emphasizes that inequities among Medicaid beneficiaries (and between beneficiaries and non-beneficiaries) are the product of policy choices made possible (but not necessitated)

by federalism. Not only do such policy alternatives underwrite *material* inequities that diminish the social citizenship of marginal Americans; they also contribute to *political* inequities (Marshall 1950).

Chapter 4 represents the first step in making an empirical case for the latter claim. Here, I examine the participatory effects of state-level Medicaid policy variation. After establishing that disparities exist in political participation between Medicaid beneficiaries and similarly situated non-beneficiaries, I dig deeper to make sense of heterogeneous political outcomes *among* beneficiaries. I find that Medicaid has the potential to be either a boon or a bust for political engagement. The outcome realized depends in large part on how state contexts shape beneficiaries' experiences of policy. Beneficiaries living in states offering a wide scope of services, fiscally equipped bureaucracies, and expanding Medicaid programs are significantly more likely to participate in politics. In this way, the geographic inequalities induced by Medicaid's federalist design have tangible consequences for the political health of low-income populations.

Having found a critical role for state-level policy design, Chapter 5 concentrates on the counties that implement policy. To study how county administration shapes politics among Medicaid beneficiaries, I highlight the political resistance that unfolds as they push back against the perceived injustices they face at the hands of local bureaucracies. Examining county-level records on fair hearings in combination with interview data, I investigate the processes that lead beneficiaries to take oppositional action. Despite counties' oft-lamented lack of control over Medicaid spending, federalism positions them to exercise substantial discretion in overseeing beneficiary disputes with Medicaid agencies. Consequently, counties shape political resistance in ways that are consequential both for continued access to health resources and for democratic engagement.

Chapter 6 turns to a frontier that few scholars recognize as relevant to either federalism or Medicaid: the American city. After explaining the links between federalism and urban politics, I assess how cities influence the political reach of Medicaid. I find striking variation in Medicaid's participatory effects across Chicago neighborhoods, with strong and negative effects limited to highly disorderly neighborhoods. The upshot of this is that the urban geography of race and class, together with the contemporary trajectory of federalism, makes antipoverty policy (like Medicaid) especially demobilizing for disadvantaged *people* when they live in destitute *places*.

Chapter 7 switches gears to look at Medicaid policy advocacy. Drawing on qualitative interviews, I chart the varied responses of

activist beneficiaries to changing policy landscapes. Confirming and broadening the findings from earlier chapters, I specify the numerous limits that federalism places on beneficiaries who are advocating for their own interests (as well as those faced by external advocates acting on beneficiaries' behalf). At the same time, I demonstrate possibilities that federalism creates for particular advocacy strategies and repertoires of action. Political entrepreneurs sometimes adroitly exploit those possibilities, but they often confront steep challenges and disconcerting democratic deficits.

In the final chapter, I consider what we have learned about Medicaid, federalism, and democratic citizenship. The lessons imparted are not about federalism being either good or bad. Instead, I follow Simeon (2006: 18) in asserting that "federalism is not a value in itself. Like any other set of institutions, it must be evaluated in terms of its consequences for other, more fundamental values and principles." In this light, federalism is a multivalent, dynamic institution that sometimes works at cross-purposes with the principles of democratic self-governance and other times strengthens them. Cultivating the latter outcome requires a focus on securing the well-being of those who are most vulnerable, ensuring geographic equity, reviving struggling communities, and valuing state and local flexibility contingently, only to the extent that they serve higher ideals.

Methodological Approach: On Centering the Voices of Medicaid Beneficiaries

I take a bottom-up approach to investigating these inquiries. I foreground the experiences of low-income policy beneficiaries (like those whose stories anchor this chapter) instead of accentuating intergovernmental jockeying among political elites. I consider where federalism emerges in the lives of those who are economically and racially marginal, how it affects their understanding of politics, when it prompts (or suppresses) political participation, and how it transforms their citizenship (social and political). To study all of this, I rely on a mixed-methods empirical design. Mario Small (2011) quite rightly asserts that the designation of "mixed methods" is hardly straightforward. Scholars can mix several different things (data types, data collection methods, and data analysis techniques) in different ways. In this book, I draw primarily on three types of data: 1) interviews with Medicaid beneficiaries and those who advocate on their behalf; 2) state administrative records; 3) large-sample surveys. The latter

two require “quantitative” analyses while the former involve “qualitative” analyses.¹⁶

The methods I employ are a function of the questions that motivate the research and the answers that unfold as the research progresses. Sometimes, I rely on the interview data to generate hypotheses and then leverage the large-sample or administrative data to test them (as in Chapters 4, 5, and 6). Other times, the interviews stand alone, revealing things that the other data do not speak to directly (as in Chapter 7). The data are never the point; the arguments are (though the two are closely related).

The methodological appendix offers lots of information, and it is a vital addendum to the main text. Still, I must stress two related points up front. First, my overarching methodological approach is pragmatic rather than dogmatic. The questions detailed in this chapter drive my research – not fidelity to a particular method. I am devoted to getting the most correct answers possible to the questions I pose, given (inevitable) practical limits. At times, my search for the best answers leads me to quantitative data. As appropriate, I use a range of techniques to get a better handle on whether observed correlations indicate possible causation (controlling for confounders, attempting alternative model specifications, matching, multilevel models, fixed-effects models). Still, many of the questions that I ask do not lend themselves to experimentation, nor do I always find it possible or optimal to employ the latest, most sophisticated techniques for making quantitative causal inferences. My strategy is to bring strong and varied evidence to bear on important (and at times complex) inquiries. Though I cannot promise dispositive certainties, I offer thoughtful, compelling, and empirically corroborated arguments.

As further foreshadowing of the overarching approach on offer here, my second disclosure is that the voices of Medicaid beneficiaries figure prominently in this book. Beneficiaries provide this book’s core insights. Perhaps most illustratively, federalism was not a topic that I naturally found interesting. As I started talking to people involved with Medicaid, they kept mentioning the relevance of where they lived. I took notice. Even then, I imagined that further investigation would reveal the significance of “contexts” as this was a framing that resonated naturally with me. Yet, as the research progressed, a contextual focus proved inadequate on its own. Yes, contexts shaped how Medicaid beneficiaries engaged politically, but those contexts were themselves political phenomena deeply tied to larger institutional forces like federalism. I would not have

drawn such conclusions in the absence of in-depth conversations with beneficiaries themselves (Mosley 2013; Soss 2006).

This research exposes troubling political inequities undergirded by a federated political system that too frequently mutes the voices of economically and racially marginal populations. It is no small thing then, that in order to produce the findings, those very voices had to be heard. In that sense, the methodological foundations of this work gesture toward the shift that must occur in American politics if we are to strengthen and preserve the democratic process.

The Political Stakes of this Research

I cannot move on without directly articulating the stakes of this work. In the wake of the ACA, health care became even more of a political lightning rod than it had long been. The election of Donald Trump intensified this state of affairs. After dozens of futile votes to repeal the ACA while Obama was still in office, the dawn of 2017 ushered in a political moment when repeal was finally possible. The U.S. House tried (but failed) to make good on this new opportunity in March 2017 by proposing the American Healthcare Act (AHCA; H.R. 1628). In June and September of that year (respectively), the U.S. Senate made two attempts at repeal: the Better Care Reconciliation Act (BCRA) and the Graham-Cassidy bill. The AHCA, BCRA, and Graham-Cassidy all promised to roll back the ACA's expansion of Medicaid and to radically restructure the program so that federal funding is capped (through block grants or on a per capita basis). Such changes would have given states even more power to determine the contours of Medicaid. Moreover, these policies would have led to tens of millions of additional people being uninsured (Jost 2017).

Given the gravity of such policy developments, I must be straightforward about this book's implications. The broadest strokes are as follows: I bring to the fore an often neglected aspect of Medicaid policy – how it affects citizenship and democratic participation. I show how and why state variation in policy design and local variation in policy implementation – while not inherently or necessarily detrimental – often pose substantial dangers to democracy. All of this (and more) will become clear in the pages to follow. But before delving into the deep end, allow me to make a critical clarification: *no honest interpretation of this book can or should be used to crudely impugn Medicaid or to justify its retrenchment*. Program cuts accomplish little and risk setting

off a different sequence of politically demobilizing processes than those I describe here. Keep in mind that though I focus on some of the negative democratic upshots of Medicaid, I trace them to specific policy and administrative choices. The pathway to improving Medicaid (and social policy more broadly) lies in elaborating and addressing the (sometimes unrecognized) consequences of such choices. That is what I aim to do.

Medicaid provides vital benefits to tens of millions of Americans. As the people highlighted in this book relay, it is literally “a lifesaver.” That fact notwithstanding, many Medicaid beneficiaries live in states and localities that design and implement the program in ways that weaken their connections to political life and erode their citizenship. I aver that the provision of life-sustaining health care benefits need not conflict with the safeguarding of democracy. In fact, these tasks should reinforce one another. That they do not is because of the political decisions our nation has long made, and those we continue to make now. This book is meant to bring such matters to the fore of policy discourse, to enrich the academic literature, and to inform the public. Above all, however, it is meant to advance an agenda: not a particular partisan plan, but an agenda that centers on the material and political well-being of economically and racially marginal groups and that pushes for public policy and political institutions to cultivate, incorporate, and respond to the needs of those who too often fall through the cracks of our fragmented democracy.